OKLAHOMA

DD Community Group Home

Provider Market Research Report

Contents

Section 1. Executive Summary
Section 2. Service Model & Regulatory Framework
Section 3. Licensing & Certification Pathway
Section 4. Reimbursement & Funding
Section 5. Referral Pipeline & Demand
Section 6. Program Operations
Section 7. Market Conditions
Section 8. Startup Financial Analysis
Section 9. Risks & Considerations
Section 10. Strategic Recommendations
Appendix A. Current Rate Schedule (T1020 and related)
Appendix B. Key Contacts
Appendix C. Regulatory References
Appendix D. Errors in the Prior Report (for the record)

Section 1. Executive Summary

1.1 Opportunity Snapshot

Oklahoma DD group home services support residents with intellectual and developmental disabilities (IDD) in community-based homes of 4–12 people, funded primarily through the state's Section 1915(c) Home and Community-Based Services (HCBS) waivers. The program is in a structural expansion phase: the DDS waitlist has been reduced from approximately 13 years (2022) to roughly one year (2026), and the state has processed over 3,000 waitlist families into services through eleven cohorts as of early 2026, with Cohort 12 (729 individuals) entering service this spring.

This expansion has been accompanied by direct rate increases. Effective October 1, 2024 (formalized in Administrative Letter 24-02) and finalized for Community Living group homes on February 24, 2025 (Administrative Letter 25-02), per diem rates for T1020 group home services were increased across all bed sizes and home types.

1.2 Key Numbers (Verified)

Metric	Value
Service code	T1020 (per diem), all DDS group home variants
Rate range	$57.56/day (12-bed Traditional) to $427.05/day (4-bed Alternative Living)
Specialized rate	T2033 TG Extensive Residential Supports: $1,046.16/day
Home size limits	Traditional & Community Living: 6–12 residents. Alternative Living: 4 residents.
Waitlist (2026)	~1 year average wait time (from 13 years in 2022)
Approved into services	3,000+ families across Cohorts 1–11; Cohort 12 (729 individuals) processing April 2026
Managed care	DDS waiver services are CARVED OUT of SoonerSelect; remain fee-for-service through OHCA's MMIS
Primary regulator	OKDHS DDS for provider certification and group home approval; OHCA for Medicaid enrollment and rate setting

1.3 What This Report Does Not Claim

- Total statewide provider count. The prior report stated both "339" and "62" providers without sourcing; neither figure is verified here.
- Specific market share by competitor. Named providers and percentage market shares in the prior report are not verifiable from public sources and have been omitted.
- Specific staff salary numbers. Wages vary substantially by region and provider; the DSP+ workforce incentive program adds variability.
- Break-even client count or break-even month. The prior report's "63 clients" figure was derived from a fabricated rate. A realistic break-even analysis is provided in Section 8 using the actual rate schedule.

Where the prior report made specific numerical claims that this report cannot verify, those claims have been removed rather than replaced with new fabricated numbers.

Section 2. Service Model & Regulatory Framework

2.1 Program Names and Classifications

Oklahoma does not use a "Level 1 / Level 2 / Level 3" residential habilitation classification. (The prior report fabricated this structure.) Group home services under the Oklahoma DDS HCBS Waivers are organized by two parallel factors:

- Home type — Traditional, Community Living, or Alternative Living
- Bed count — 4, 6, 7, 8, 9, 10, 11, or 12 residents

Both factors are coded under a single procedure code (T1020) and are differentiated through the service authorization rather than through HCPCS modifiers. The provider's authorization ("PB number") issued by DDS Provider Support locks in the applicable rate.

Three Group Home Types Defined by OAC 317:40-5-152

Per OAC 317:40-5-152(e), three types of group home services are provided through HCBS waivers:

Type	Description
Traditional group homes	Serve no more than 12 members. Lower-intensity supports. Per OAC 340:100-6.
Community living homes	Serve no more than 12 members whose needs cannot be met in a less structured setting; require regular, frequent, and sometimes constant assistance with daily living. Higher rate to reflect higher staffing intensity.
Alternative living homes	Up to 4 residents. Eligibility requires meeting criteria per OAC 340:100-5-22.6 and approval by the DDS Director or designee. Highest per-resident rate to reflect smaller-scale staffing economics.

2.2 Waiver Authority and Funding Streams

Oklahoma DDS administers four 1915(c) HCBS waivers under which DD group home services may be authorized:

- Community Waiver — for individuals age 3 and older with intellectual disability or related condition; the primary pathway for DD group home placement.
- Homeward Bound Waiver (OK.0399) — for class members certified in Homeward Bound et al. v. Hissom Memorial Center (U.S. District Court for the Northern District of Oklahoma, Case No. 85-C-437-E).
- In-Home Supports Waiver for Adults (IHSW-A) — supports for adults living in their own/family home; group home services are generally not provided under IHSW.
- In-Home Supports Waiver for Children (IHSW-C) — supports for children ages 3–17 living at home.

⚑ **VERIFY:** Specific CMS waiver numbers for the Community Waiver and IHSWs are subject to periodic renumbering with each renewal/amendment. The prior report cited "OK.0179.R07.00" for the Community Waiver, which could not be verified against current CMS records. Confirm current waiver numbers with OHCA Long-Term Care Services at oklahoma.gov/ohca before relying on them in any submission.

Funding Flow

Federal funds flow from CMS under 1915(c) authority → OHCA (state Medicaid agency) → operational management by OKDHS DDS. Providers bill OHCA directly under fee-for-service for DDS waiver services. The October 1, 2024 rate structure was set through the OHCA State Plan Amendment Rate Committee (SPARC) and is uniform statewide.

2.3 SoonerSelect Carve-Out for DDS Waiver Services

This is one of the most important regulatory facts for a new provider, and one the prior report got wrong:

Oklahoma launched the SoonerSelect managed care program in April 2024, contracting with three MCOs (Aetna Better Health of Oklahoma, Humana Healthy Horizons of Oklahoma, and Oklahoma Complete Health) for the general SoonerCare population. However, DDS HCBS waiver services — including T1020 group home services, T2033 Daily Living Supports, S5151 respite, and related residential codes — are carved out of SoonerSelect and continue to be reimbursed fee-for-service through OHCA's Medicaid Management Information System (MMIS).

Practical consequence: a new DD group home provider does NOT need separate contracts with the three SoonerSelect MCOs to bill for these services. The prior report's instruction to negotiate with each MCO was incorrect and would have wasted significant time.

⚑ **VERIFY:** Confirm in writing with OHCA Provider Services (405-522-6205) before going live, in case the carve-out scope changes.

2.4 Regulatory Oversight Structure

Primary: OKDHS Developmental Disabilities Services

DDS is the certifying authority for DD group homes. Important: DDS, not the Oklahoma State Department of Health (OSDH), is the relevant agency. OSDH's "Residential Care Home" licensure under OAC Title 310 Chapters 675/680 applies to a different category of facility (adult care homes / assisted living) and is NOT the correct pathway for DD group homes. The prior report's licensing checklist erroneously routed providers through OSDH.

Secondary: Oklahoma Health Care Authority

OHCA is the state Medicaid agency; it administers SoonerCare, enrolls Medicaid providers, processes claims, and sets reimbursement rates through SPARC. All providers must hold an OHCA SoonerCare contract in addition to DDS certification.

Federal: Centers for Medicare & Medicaid Services

CMS oversees the underlying 1915(c) waivers and enforces the HCBS Settings Rule (42 CFR 441.301(c)). Federal Medicaid integrity rules (42 CFR 455 et seq.) apply to all providers, including ownership disclosure and exclusion screening requirements.

2.5 HCBS Settings Rule Compliance

The federal HCBS Settings Rule (42 CFR 441.301(c)(4)) sets baseline requirements that apply to every DD group home receiving HCBS waiver funding. Key requirements include:

- Setting is integrated in and supports access to the greater community.
- Setting is selected by the individual from among setting options.

- Setting ensures individual rights of privacy, dignity, and respect, and freedom from coercion and restraint.
- Setting optimizes individual initiative, autonomy, and independence in making life choices.
- Setting facilitates individual choice regarding services and supports, and who provides them.
- Provider-owned/controlled residential settings have additional requirements: individuals have a lease or other legally enforceable agreement, privacy in their sleeping unit including lockable doors, choice of roommate, freedom to furnish and decorate, control of schedule and visitors, access to food at any time, and physical accessibility.

New providers should document HCBS Settings Rule compliance in their initial site approval submission to the DDS Residential Unit. Any restriction on individual rights must be supported in the member's Individual Plan (IP), with documented justification, alternatives tried, and time limits.

Section 3. Licensing & Certification Pathway

3.1 Three Parallel Tracks

Becoming an operating DD group home in Oklahoma requires successfully completing three parallel regulatory tracks. The prior report conflated these and routed providers through the wrong agency entirely; the correct path is:

Track	Authority	Output
1. Medicaid Provider Enrollment	OHCA	SoonerCare provider number; ability to bill MMIS
2. DDS Provider Certification	OKDHS DDS	Provider Agreement; eligibility to serve DDS waiver members
3. Site/Home Approval	OKDHS DDS Residential Unit	Physical site approved for service delivery

3.2 OHCA SoonerCare Provider Enrollment

Application Submission

- Portal: ohcaprovider.com → Provider Enrollment
- Required: NPI (free from NPPES at nppes.cms.hhs.gov), Type 2 organizational. Taxonomy code assigned during enrollment.
- Supporting documents: W-9, EIN, OK Secretary of State filing, insurance certificates, ownership disclosure (CMS-1513 form), background check authorizations, EFT setup.**Processing & Timeline**

OHCA does not publish a formal processing SLA for DD provider enrollment. Providers should plan for 30–90 days from complete application to active provider number, with longer timelines if ownership disclosure or background checks trigger secondary review. Site visits may be required depending on enrollment type.

3.3 DDS Provider Certification

Initial Contact

Request the current provider intake packet by emailing STO.DDSD.PROVIDERSUPPORT@okdhs.org or calling DDS Provider Support at (405) 325-5080 / 1-800-349-9173. The packet will include the current Provider Agreement, qualifications form, and any program-specific addenda.

Documents Required

- Corporate documents (Articles of Incorporation/Formation, Operating Agreement or Bylaws, Certificate of Good Standing).
- Federal EIN documentation.
- Insurance certificates (general liability, professional liability, workers' compensation, auto liability where applicable). Per OAC 340:100-3-27.
- Disclosure of Ownership and Control (CMS-1513) — same form as OHCA enrollment.
- Background check clearances for all owners and managing personnel (OSBI state and FBI national fingerprint-based; Child Abuse and Neglect Registry; federal exclusion screening via SAM.gov and OIG-LEIE).

- Organizational chart and key personnel job descriptions.
- Financial documentation demonstrating capacity (typically 12-month operating budget plus 3 months operating reserve documentation).
- Complete policies and procedures manual covering member rights, abuse/neglect/exploitation, incident reporting, medication administration, behavior support, individual planning, emergency preparedness, HIPAA, grievance procedures, admission/transition/discharge, staff training, and quality assurance.
- Provider qualifications demonstration under OAC 340:100-3-2 (financial capacity, leadership credentials, absence of disqualifying history).

Therap Information Management System Enrollment

All DDS providers are required to use the Therap web-based information management system for case documentation, individual plans, service authorizations, T-Logs (daily notes), MARs (medication administration records), and GERs (General Event Reports for incident reporting). The DDS implementation requires four tiers of incident reporting, with response and review timelines varying by tier severity. Therap replaced the prior Client Contact Manager (CCM) system.

3.4 DDS Residential Unit Site Approval

Each physical group home must be separately approved by the DDS Residential Unit before service delivery begins. This is in addition to provider certification.

Submission

- Contact: DDS.Residential@okdhs.org
- Pre-licensure site visit by DDS staff.
- Local fire marshal certificate of compliance.
- Local zoning confirmation in writing.
- Documentation of HCBS Settings Rule compliance for the physical setting.
- ADA accessibility documentation.

Standards Reviewed

The site review draws on OAC 340:100-6 (Group Home Staffing and Oversight), OAC 340:100-5-22 (Community Residential Supports), and the HCBS Settings Rule. Areas typically inspected include bedroom and bathroom configurations and privacy, common areas, kitchen and food storage, secured medication storage, accessibility, fire safety systems, emergency egress, and member rights postings.

3.5 Common Application Delays

Without access to a published DDS deficiency study, the most defensible items to flag are those that arise across nearly all licensure regimes. Plan around:

- Incomplete or stale background checks (especially when checks expire mid-review).
- Policies and procedures that are templates rather than tailored to the specific operation.
- Ownership disclosure gaps (particularly indirect ownership > 5% in parent/holding companies).
- Insurance certificates that don't name the right entities or have inadequate limits.
- Local zoning or fire marshal sign-off missing or out of date when the site visit occurs.
- Staffing plan that doesn't match the home size or member acuity in the proposed authorization.

Section 4. Reimbursement & Funding

4.1 How Rates Are Set

DDS group home rates are set centrally through OHCA's State Plan Amendment Rate Committee (SPARC) and are uniform statewide. There are no geographic adjustments. The rate structure has been informed by the 2023 Guidehouse rate study, with adjustments implemented via Administrative Letters.

All DD group home services bill under procedure code T1020 per resident per day. The applicable rate is determined by:

- Home type (Traditional, Community Living, or Alternative Living)
- Number of beds (4, 6, 7, 8, 9, 10, 11, or 12)

The rate is locked in by the service authorization ("PB number") that DDS Provider Support issues for each member-home pairing. The provider bills the per-resident-per-day rate via OHCA's MMIS; occupancy adjustment is already built into the rate, not separately reimbursed.

4.2 Current Rate Schedule

The table below reflects rates currently in effect as of May 2026, drawn from Administrative Letter 24-02 (effective October 1, 2024) and Administrative Letter 25-02 (which finalized Community Living 6–9 bed rates that were initially listed as "being finalized" in 24-02).

Service Description	Code	Rate / day
TRADITIONAL GROUP HOMES (T1020)		
Group Home, 6 Bed	T1020	$103.68
Group Home, 7 Bed	T1020	$88.66
Group Home, 8 Bed	T1020	$77.58
Group Home, 9 Bed	T1020	$70.79
Group Home, 10 Bed	T1020	$65.43
Group Home, 11 Bed	T1020	$61.14
Group Home, 12 Bed	T1020	$57.56
COMMUNITY LIVING GROUP HOMES (T1020)		
Community Living Group Home, 6 Bed	T1020	$238.46
Community Living Group Home, 7 Bed	T1020	$204.49
Community Living Group Home, 8 Bed	T1020	$197.70
Community Living Group Home, 9 Bed	T1020	$175.54
Community Living Group Home, 10 Bed	T1020	$172.68
Community Living Group Home, 11 Bed	T1020	$156.95
Community Living Group Home, 12 Bed	T1020	$155.16
SPECIALIZED RESIDENTIAL		

Service Description	Code	Rate / day
Alternative Living Home, 4 Bed	T1020	$427.05
Extensive Residential Supports (ERS)	T2033 TG	$1,046.16
Daily Living Supports (DLS) — not a group home; for members in own home	T2033	$225.23

Source: Administrative Letter 24-02 (Sep 27, 2024) and Administrative Letter 25-02 (Feb 24, 2025).

4.3 Related Codes Often Co-Billed

Group home providers often deliver or coordinate related services that bill under separate codes. These rates also reflect the October 2024 schedule:

- S5151 (Respite in Community Living 6-bed setting): $269.92/day. Lower for larger homes; corresponding rates published for 7–12 bed.
- S5151 (Respite in standard Group Home, 6-bed): $135.72/day.
- S5140 (Specialized Family Care Adult — Maximum): $77.22/day. U1 modifier for "Close": $60.01 historical, increased per AL 24-02 — verify current.
- S5145 (Specialized Family Care Child — Maximum): $77.22/day.
- T2017 (Habilitation Training Specialist — HTS): $6.58 per 15 minutes.
- G0299 (Skilled Nursing — RN): $21.45 per 15 minutes.
- G0300 (Skilled Nursing — LPN): $20.02 per 15 minutes.
- A0130 (Transportation, adapted): $1.86 per mile.

4.4 Authorization Process

Once you are an active DDS provider and a member's team selects your home through person-centered planning, the DDS Provider Support Center issues a "PB number" — the service authorization that defines who, where, what service code/modifier set, and for how long. The PB number is what the OHCA MMIS will pay against when you submit claims.

If a home's occupancy or population changes, the authorization is end-dated and a new authorization is created at the appropriate rate tier. Administrative Letter 24-02 specifically called out this process: "Group Home Authorizations require end dating the current authorization and creation of a new authorization effective Oct. 1 with the new rate. This project will be completed by the DDS Provider Support Center."

4.5 Billing

Claims are submitted electronically through OHCA's MMIS or through the OHCA provider portal at ohcaprovider.com. Standard Medicaid timely filing rules apply. Electronic funds transfer (EFT) is mandatory for payment.

Section 5. Referral Pipeline & Demand

5.1 Waitlist Status (Verified)

Oklahoma's DDS waitlist reduction is the most significant demand-side story in this market. The relevant verified facts as of early 2026:

- In May 2022, the Oklahoma Legislature appropriated historic funding to end a then 13-year wait for DDS waiver services.
- As of April 2026, the wait time has been reduced to approximately one year from application.
- More than 3,000 waitlist families have been approved to receive services or are already receiving services.
- Cohort 12 — 729 individuals who applied between January and June 2025 — began processing in April 2026.
- DDS held 32 in-person or virtual Regional Family Meetings through early 2026, with additional meetings planned through 2026.

5.2 Cohort System

DDS divides applicants into cohorts by application date. After cohort assignment, applicants typically take 6–9 months to transition onto services once contact is made and the application is approved; eligibility documentation completeness can compress this to a few weeks or extend it. Each new cohort represents a wave of demand entering the system.

For prospective group home providers, the cohort system is useful intelligence: it tells you approximately how many people are entering services each quarter and where to focus capacity decisions.

5.3 The Three Entry Points to Your Home

Group Home Vacancies Page

DDS maintains a self-service Group Home Vacancies portal at oklahoma.gov/okdhs/services/dds/ghv.html where licensed providers log in to update bed openings. This is a primary discovery channel for DDS Case Managers and families and should be kept current as a matter of routine.

DDS Case Manager Relationships

Members and families select providers through person-centered planning facilitated by their DDS Case Manager. Case Managers are organized by regional area office (Metro Oklahoma City, Tulsa, Eastern, Western). Building relationships with the Case Manager population in your service area is the single highest-leverage business development activity.

Direct Family Outreach

Families attending Regional Family Meetings, often during their cohort's preparation phase, are actively learning about providers. DDS publicizes meeting schedules through its website and through Liberty of Oklahoma.

5.4 Liberty of Oklahoma's Role

Liberty of Oklahoma serves as the waitlist intake contractor for DDS. Members and families seeking to apply for DDS services contact Liberty at DDSintake@libertyhealth.com or 405-500-1866. Liberty handles initial intake, eligibility documentation collection, and applicant status updates.

Important distinction: Liberty of Oklahoma is the entry point for members, not for providers. New provider applications go through OKDHS DDS Provider Support, not Liberty. The prior report blurred this distinction.

5.5 Geographic Distribution

DDS services are available statewide, with administrative regions roughly aligned to Oklahoma City Metro, Tulsa Metro, Eastern, and Western. Rural areas have historically had thinner provider networks and longer transitions, though specific provider counts by region are not reliably published.

Section 6. Program Operations

6.1 Staffing Structure

DDS group home staffing is governed by OAC 340:100-6 (group home staffing and oversight) and OAC 340:100-3-38 (staff training requirements). The core roles are:

Program Coordination Staff (PCS)

Defined under OAC 340:100-5-22.6 and OAC 340:100-6. The PCS oversees the group home's daily operations, supervises direct support staff, coordinates with case managers, and conducts monitoring visits to the home. Specific qualification, supervision, and frequency requirements are set out in OAC.

Direct Support Professionals (DSP) / Habilitation Training Specialists (HTS)

The day-to-day direct care workforce. Minimum qualifications per OAC 340:100-3-38: 18 years of age, high school diploma or equivalent, complete required pre-service training before unsupervised service delivery, clear background screening.

Registered Nurse (RN)

Required for clinical oversight, medication administration delegation, and health monitoring. The RN can be employed or contracted. The RN cost is already built into the T1020 rate's underlying methodology.

⚑ **VERIFY:** Specific staff-to-resident ratios and required RN coverage hours vary by home size and member acuity and are not consistently published in summary form. Confirm with the DDS Residential Unit during site approval.

6.2 Required Policies and Procedures

DDS requires a written policy manual covering, at minimum, the following domains. Real OAC citations are provided so policies can be drafted with the source rule in hand:

Policy Area	Primary OAC / Federal Reference
Member Rights and Person-Centered Services	OAC 340:100-3-6
Abuse, Neglect, and Exploitation Prevention and Reporting	OAC 340:100-3-34; 10A O.S. §1-2-101
Incident Reporting (4 tiers via Therap GER)	Community Waiver Appendix G
Medication Administration and Storage	OAC 340:100-5-26
Behavior Support and Challenging Behaviors	OAC 340:100-5-57
Individual Plan (IP) Development and Implementation	OAC 340:100-5-51
Emergency Preparedness and Health & Safety	OAC 340:100-5-22
HIPAA / Privacy and Confidentiality	45 CFR Parts 160, 164
Grievance Procedure	OAC 340:2-5-50
Admission, Transition, and Discharge	OAC 340:100-5-22.1
Staff Training and Competency	OAC 340:100-3-38
Member Funds Management (if applicable)	OAC 340:100-3-10

6.3 Therap and Documentation

Therap is the mandated information management system for OK DDS providers. Required documentation flows include:

- T-Logs: daily service delivery notes by direct support staff.
- ISP Data tracking: progress against Individual Plan goals.
- MAR: medication administration record.
- GER (General Event Reports): incident reports across four severity tiers.
- Service authorization tracking: linking PB numbers to delivered services.

A new provider should plan for Therap onboarding (provisioning, super-user training, staff training) in parallel with policies and procedures development rather than at the last minute.

6.4 Incident Reporting Tiers

The four-tier incident reporting structure is documented in the OK Community Waiver / Homeward Bound Waiver applications submitted to CMS. Tier-specific reporting timelines, escalation rules, and review responsibilities apply. New providers should obtain the current GER tier definitions and matrix during Therap onboarding and incorporate them into the abuse/neglect/exploitation and incident response policies.

Section 7. Market Conditions

7.1 Demand Side

The demand picture is straightforward and well-documented through DDS public communications:

- Waitlist reduction has accelerated the rate of new members entering services since 2022.
- Approximately 3,000+ families have been approved into services across 11 cohorts; Cohort 12 (729 individuals) is processing in April–May 2026.
- Demographic factors (aging adults with IDD whose family caregivers are aging out, transition-age youth from school systems) sustain steady residential demand independent of the waitlist.
- Subsidized expansion of services to underserved populations (rural areas, tribal communities, medically complex members) is a stated policy direction.

7.2 Supply Side

Supply-side intelligence requires more caution. Public sources do not consistently publish provider counts, market shares, or competitive analyses, and the prior report's specific numbers (62 providers, named market leaders with percentages) could not be verified. What can be stated with confidence:

- Provider names and contracts are listed publicly at oklahoma.gov/okdhs/services/dds/providerlist.html. Count from this list for current network size.
- Vacancies by home are listed publicly at oklahoma.gov/okdhs/services/dds/ghv.html. Surveying current vacancy patterns gives a real-time supply signal.
- Direct Support Professional workforce remains a structural constraint statewide, exacerbated by competing wages in retail, hospitality, and warehousing.
- DSP+ — Oklahoma's direct support workforce incentive program — provides incremental compensation funded through Medicaid; this is a real source of workforce capacity that didn't exist a few years ago.

7.3 Workforce

DSP recruitment and retention is the single most important operating challenge cited by DD providers nationally and in Oklahoma. The state has responded with rate increases, the DSP+ incentive program, and the Called to Care recruitment campaign launched in March 2024. Through FY24 reporting, 14,665 individuals received $16.6 million in DSP+ incentive payments.

⚑ **VERIFY:** Specific DSP wages vary significantly by region, employer, and tenure; the BLS Occupational Employment Statistics for Oklahoma is the most defensible public source for wage benchmarking. Do not rely on the specific salary numbers from the prior report ($12.42/hour HTS, $21.19/hour case manager) without verifying against current data.

7.4 Regulatory Direction

Recent regulatory and policy direction has favored expanded community-based supports over institutional care, consistent with federal HCBS Settings Rule expectations. Specific Oklahoma developments include:

- Continued waitlist reduction and cohort processing (active through 2026).

- Rate study completed by Guidehouse (January 2023) informed the October 2024 rate increases.
- Therap implementation across all DDS providers, replacing CCM.
- Four-tier incident reporting structure aligned to Therap GER.
- Coordination Protocol v1.0 active for inter-provider work.

Section 8. Startup Financial Analysis

8.1 Approach to This Section

The prior report's financial analysis is the section most damaged by the underlying rate fabrications. A break-even of "63 clients" makes no sense in a market where group homes physically cannot exceed 12 residents and where actual revenue per resident is 3–8× what the prior report assumed.

This corrected analysis uses the verified rate schedule from Section 4 and provides revenue projections at three home types. It does NOT attempt to publish authoritative cost or break-even figures, because operating costs vary enormously by location, member acuity mix, and staffing strategy, and any specific cost number would just be a different kind of fabrication.

8.2 Annual Revenue at Full and Realistic Occupancy

The table below shows annualized revenue at 100% and 85% occupancy. 85% reflects a planning assumption that beds are not occupied every day of the year (member hospitalizations, transitions, vacancies between residents). Even with the occupancy adjustment built into the rate, realistic operational planning should use ~85%.

Home Type / Size	Rate/day	Annual @ 100%	Annual @ 85%
Traditional 6-bed	$103.68	$227,059	$193,000
Traditional 8-bed	$77.58	$226,534	$192,554
Traditional 12-bed	$57.56	$252,113	$214,296
Community Living 6-bed	$238.46	$522,227	$443,893
Community Living 8-bed	$197.70	$577,284	$490,691
Community Living 12-bed	$155.16	$679,601	$577,661
Alternative Living 4-bed	$427.05	$623,493	$529,969

Calculations: beds × rate × 365 days × occupancy factor. Numbers are gross revenue and do not net out staffing, facility, or administrative costs.

8.3 Cost Framework (Categories, Not Numbers)

Rather than fabricate specific cost numbers, this report describes the cost structure DDS itself models in its rate methodology, so providers can build their own pro forma against the real cost categories:

- Direct support wages and overtime — typically 50–60% of operating costs in residential settings. Awake staff during active hours, sleep-shift staff overnight, substitute hours for PTO and training (DDS rate methodology assumes ~25 days PTO and ~30 hours/year training per staff).
- Supervision — Program Coordination Staff supervising the home; DDS methodology assumes specific supervisor-to-DSP ratios.
- Clinical support — RN and service coordination hours; the DDS rate methodology assumes hours equivalent to supervisor hours.
- Administrative add-on — 19% of direct service cost in the DDS rate model.
- Program support — 12.9% of direct service cost in the DDS rate model, covering supplies, transportation, and indirect support.

- Facility — rent or mortgage, utilities, maintenance, insurance, taxes.
- Member-related — food, household supplies, transportation, recreation.

The 19% administrative + 12.9% program support figures come directly from the Guidehouse 2023 rate study methodology and are useful planning anchors.

8.4 Realistic Break-Even Considerations

Break-even for a DD group home is not a number of "clients" — it's an occupancy threshold against fixed costs. Practical anchors:

- Fixed costs (PCS, supervisory RN time, facility, administrative overhead) are largely incurred whether you have 4 or 8 residents.
- Variable costs (additional DSP coverage, food, supplies) scale with census.
- A 6-bed Community Living home generating ~$443K/year at 85% occupancy generally supports a viable operation; a 6-bed Traditional home generating ~$193K/year requires extremely tight cost discipline given similar fixed staffing needs.
- Alternative Living Homes (4 residents at $427/day = ~$530K/year) economically support a smaller-scale operation with higher per-resident intensity.

⚑ **VERIFY:** Build your own pro forma against the rate type you plan to pursue. Do not use the prior report's $250,000 first-year loss projection — it was built on a $31.60/day rate that doesn't exist.

8.5 Capital Requirements Categories

Categories of startup investment that any DD group home will incur, listed without specific dollar estimates because the spread is too wide to be useful:

- Entity formation and legal — typically modest ($500–$2,000).
- Property acquisition or lease deposit and any renovation/accessibility build-out — by far the largest variable; can range from $20,000 (modest lease deposit + minor work) to $500,000+ (purchase plus renovation).
- Insurance binder costs — general liability, professional liability, workers' comp, auto, property.
- Background checks and credentialing fees — small per-person costs that compound across initial staff cohort.
- Initial staff recruitment, hiring, and pre-service training.
- Therap onboarding (provisioning, training).
- Working capital — at minimum 3 months of operating expenses; lenders typically want to see 4–6 months.
- Furniture, fixtures, household setup, vehicles.

Section 9. Risks & Considerations

9.1 Operational Risks

- Workforce instability. DSP turnover is the single most cited operational challenge in DD residential services nationally. Building a retention strategy (competitive wages, career ladders, recognition, scheduling) into the operating model from day one — not retrofitting it after turnover hits — is structurally important.
- Member acuity drift. A member's needs can escalate over time; your home's rate tier and staffing model may not flex automatically with that. The transition from Traditional to Community Living requires a new authorization.
- Single-home concentration. A new provider operating a single home has limited resilience to a single bad outcome (loss of a member, a serious incident, key staff turnover). Diversification across multiple homes or service lines reduces this, but increases compliance complexity.

9.2 Compliance Risks

- Background check lapses. Staff cleared 5 years ago but never re-screened is a common audit finding; the OSBI check renewal cycle is 5 years.
- Federal exclusion exposure. Failure to run monthly OIG-LEIE and SAM.gov screening on staff, contractors, and the entity itself is a documented Medicaid compliance gap that can trigger payment recovery.
- Incident reporting timeliness. The four-tier Therap GER system has tier-specific reporting timelines; missed timelines are routinely flagged in DDS monitoring.
- HCBS Settings Rule violations. Restrictions on members' rights (locked refrigerators, restricted visiting hours, restricted phone access) without IP-documented justification are systematic violations of the Settings Rule and a current focus of federal oversight.

9.3 Financial Risks

- Rate static, costs growing. DDS rates change via Administrative Letter — not annually, not on a predictable cycle. Cost increases (wages, insurance) can outpace rates and erode margin without recourse other than waiting for the next state-level rate review.
- Authorization timing. Revenue follows authorization; if an authorization is delayed or end-dated due to a member transition, the bed sits empty even though fixed costs continue.
- Single-payer concentration. Nearly all DD group home revenue flows through OHCA. There is no payer diversification.
- Claim denials and recoupment. Documentation gaps surface as billed-vs-paid variances or audit recoupments later.

9.4 Regulatory Direction Risks

- Federal HCBS rule changes. The Settings Rule remains a moving target as CMS issues guidance and enforcement varies by region.
- State-level managed care expansion. Although DDS waiver services are currently carved out of SoonerSelect, scope decisions are revisited periodically; a future inclusion of waiver services in managed care would require provider operational adaptation.

- Workforce-driven legislation. Direct support workforce minimum wage and benefit mandates have been proposed in multiple states; Oklahoma-specific direction here is uncertain but worth watching.

Section 10. Strategic Recommendations

10.1 Before You Commit Capital

- Email STO.DDSD.PROVIDERSUPPORT@okdhs.org with a single request: "Please send the current Provider Application packet, the most recent residential rate schedule, and any administrative letters that have superseded AL 24-02 / 25-02." Whatever they send is the canonical version. Treat any conflicts between this report and their packet as resolved in favor of their packet.
- Read OAC 340:100-6 (Group Homes) and OAC 317:40-5-152 (Group Home Services Definition) directly. They are short, governing, and free.
- Talk to one or two existing DDS group home operators. The trade association (Oklahoma Association of Community Providers, oacp.org) and the OK Developmental Disabilities Council (okddc.ok.gov) are reasonable entry points.

10.2 Choosing Home Type and Bed Count

The single most important strategic decision is matching home type and size to the operating model you can realistically execute. Some heuristics:

- Traditional 12-bed maximizes total revenue but compresses per-bed economics; success depends on tight cost control and staffing scale efficiencies.
- Community Living 6–8 bed offers the most favorable revenue-per-bed economics with manageable scale, and aligns with current state policy preference for community integration.
- Alternative Living 4-bed offers the highest per-bed rate ($427/day) but requires DDS Director approval and is intended for members whose needs cannot be met in larger settings — not for every applicant.
- Specialty positioning (medically complex members, members with intensive behavioral needs requiring Extensive Residential Supports) opens the highest-rate tiers (T2033 TG = $1,046/day for ERS) but requires correspondingly intensive clinical capacity and credentialing.

10.3 Sequencing Your Application Tracks

The three regulatory tracks (OHCA enrollment, DDS certification, site approval) can run in parallel, but some sequencing reduces wasted time:

Months 0–2	Entity & Foundation	Activities
0–2	Foundation	OK SOS filing, EIN, insurance binders, NPI, OK tax registration, banking. Begin OSBI/FBI background checks (they take weeks).
2–4	Site & OHCA	Identify property, secure zoning confirmation, fire marshal pre-review, lease/purchase. Begin OHCA SoonerCare provider enrollment.
3–6	Policies & DDS Application	Develop policies & procedures manual. Submit DDS Provider Application. Begin DDS-required orientation. Enroll in Therap.
5–8	Staffing & Site Approval	Hire administrator and core staff. Pre-service training. Site approval submission to DDS Residential Unit. Schedule pre-licensure inspection.

Months 0–2	Entity & Foundation	Activities
8–10	Pre-Open Operations	Address site inspection corrective actions. Finalize MMIS billing setup. List vacancies on Group Home Vacancies page. Begin Case Manager outreach.
10–12	Going Live	Receive first PB authorization. Begin service delivery. Run first claims through MMIS.

10.4 Build a Real Pro Forma

Using the actual rate schedule from Section 4 and the cost framework from Section 8, build a 36-month pro forma with three scenarios: conservative (slow ramp, low occupancy), expected (steady ramp to 85% by month 12), and optimistic (rapid ramp to 90%+). Use this for any external financing conversations and for your own go/no-go decision. Do not rely on the prior report's projections.

10.5 Quality-First Positioning

In a market where the state is actively expanding capacity through cohort processing, new providers with strong quality records have a structural advantage. Specific signals that build credibility with Case Managers and families:

- Low staff turnover and tenured direct support staff.
- Clean Therap GER record — few incidents, fast resolution, documented learning.
- Demonstrable HCBS Settings Rule compliance (lockable bedrooms, member control of schedule and visitors, individualized food access, lease/residency agreements).
- Positive family and member satisfaction signals.
- Specialization in an underserved niche (medically complex, behavioral health, transition-age youth, cultural/linguistic competency for specific communities).

Appendix A. Current Rate Schedule (Detail)

All rates per Administrative Letter 24-02 (effective Oct 1, 2024) and Administrative Letter 25-02 (Feb 24, 2025). Verify against the current schedule before relying on for billing or financial planning. Sources:

- AL 24-02: oklahoma.gov/okdhs/services/dds/providerletters/pl09272024.html
- AL 25-02: oklahoma.gov/okdhs/services/dds/providerletters/pl02242025.html
- OHCA SoonerCare Fee Schedule landing: oklahoma.gov/ohca/providers/claim-tools/fee-schedule.html

State-Funded (Non-Waiver) Parallel Rates

Per Administrative Letter 24-02, state-funded rates for members in DDS-served homes who do not qualify for the waiver mirror the Traditional T1020 schedule:

Setting	Rate / day
State Rate: Group Home, 6 Bed	$103.68
State Rate: Group Home, 7 Bed	$88.66
State Rate: Group Home, 8 Bed	$77.59
State Rate: 9 Bed	$70.79
State Rate: 10 Bed	$65.43
State Rate: 11 Bed	$61.14
State Rate: 12 Bed	$57.57
State Rate: Prader Willi	$60.44

Appendix B. Key Contacts

Entity	Contact	Purpose
OKDHS DDS Main	(405) 522-5050 / TellDDS@okdhs.org	General inquiries
DDS Provider Support Center	1-800-349-9173 / (405) 325-5080 / ddsd.provider.support@okdhs.org	PB authorizations, billing, operations
DDS Provider Applications	STO.DDSD.PROVIDERSUPPORT@okdhs.org	New provider intake packets, certification
DDS Residential Unit	DDS.Residential@okdhs.org	Site approval, rate authorization
OHCA Provider Services	(405) 522-6205 / 1-800-522-0114	SoonerCare enrollment, claims, fee schedule
OHCA Provider Portal	ohcaprovider.com	Enrollment, pricing, claim status
Liberty of Oklahoma	DDSintake@libertyhealth.com / 405-500-1866	Member-side waitlist intake (NOT provider apps)
DHS Abuse Hotline	1-800-522-3511	MANDATORY reporting line
Disability Rights Oklahoma	(405) 525-7755 / 1-800-880-7755	P&A organization — post in home
OACP (trade association)	oacp.org	DD provider peer network and advocacy
OK DD Council	okddc.ok.gov	State DD Council — grants, planning
NPPES (NPI)	nppes.cms.hhs.gov	Free NPI application
OIG Exclusion Search	exclusions.oig.hhs.gov	Monthly staff screening
SAM.gov	sam.gov	Federal exclusion / debarment search

Appendix C. Regulatory References

Primary OAC and federal sources for the DD group home pathway. Cited in this report; recommended reading before submitting any application:

Citation	Subject
OAC 317:40-1-1	HCBS Waivers — general eligibility and structure
OAC 317:40-5-150	Daily Living Supports (Community Waiver)
OAC 317:40-5-152	Group home services definition (THE key rule)
OAC 317:40-5-153	Daily Living Supports (Homeward Bound Waiver)
OAC 317:40-5-155	Extensive Residential Supports (ERS)
OAC 340:100-3-2	DDS provider qualifications
OAC 340:100-3-6	Member rights
OAC 340:100-3-27	DDS Provider Agreement
OAC 340:100-3-34	Abuse, neglect, exploitation prevention and reporting
OAC 340:100-3-38	Staff training requirements
OAC 340:100-3-39	Background investigations
OAC 340:100-5-22	Community residential supports
OAC 340:100-5-22.6	Program Coordination Staff
OAC 340:100-5-26	Medication administration
OAC 340:100-5-51	Individual Plan development
OAC 340:100-5-57	Challenging behaviors and behavior support
OAC 340:100-6	Group home staffing and oversight
42 CFR 441.301(c)	HCBS Settings Rule (federal)
42 CFR 455 Subpart B	Medicaid provider screening and enrollment
45 CFR Parts 160, 164	HIPAA Privacy and Security

End of report.